



CCTV Monitoring in Mental Health Areas SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health (MH) Division, Inpatient Areas, Community Mental Health Services (CMHS) excluding Wungen Kartup and Safe Haven	Medical, Nursing, Allied Health, Security and Facilities Management staff

Purpose

This standard operational procedure (SOP) aims to provide guidance for the appropriate use, access and disclosure of closed circuit television (CCTV) monitoring, footage and data and associated video and audio monitoring devices.

Links to relevant documents

- [East Metropolitan Health Service \(EMHS\) Appropriate access and disclosure of Closed Circuit Television \(CCTV\) footage and other security data Policy](#)
- [EMHS Use of Video Monitoring in Mental Health Inpatient Units Policy](#)
- [EMHS Mental Health Advocates Policy](#)

Definition

CCTV	Visual monitoring with the use of one or more video cameras that are installed in specific places for the purpose of ensuring the safety of consumers and others. Cameras are positioned in designated areas, e.g. entrances, corridors, isolated areas, courtyards, outdoor areas and seclusion rooms, where applicable.
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Risk

The privacy and dignity of consumers may be compromised.

Principles

- CCTV devices are one of several strategies available to support safety and security
- Staff are to ensure that video monitoring in consumer areas complies with legislative and ethical requirements
- CCTV monitoring is not a substitute for personal interactions and relationships with consumers
- Where possible, consumer privacy and dignity is not to be compromised by the use of CCTV monitoring, for example, CCTV will not monitor bathrooms and bedrooms
- Display screens are not to be clearly visible to visitors, members of the public or consumers
- **Only staff with direct clinical responsibility** are to undertake monitoring using the screen. Security staff (excluding Transitional Care Unit (TCU)) can view and monitor all screens with the exception of seclusion. Refer to **EMHS Use of Video Monitoring in Mental Health Inpatient Units Policy, EMHS Appropriate access and disclosure of Closed Circuit Television (CCTV) footage and other security data Policy**.

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Version: 4.0

Endorsed by: Mental Health Division Co-Director 19/12/2025

Authorised by: Clinical Practice Committee 15/01/2026

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CoN, Mental Health Division

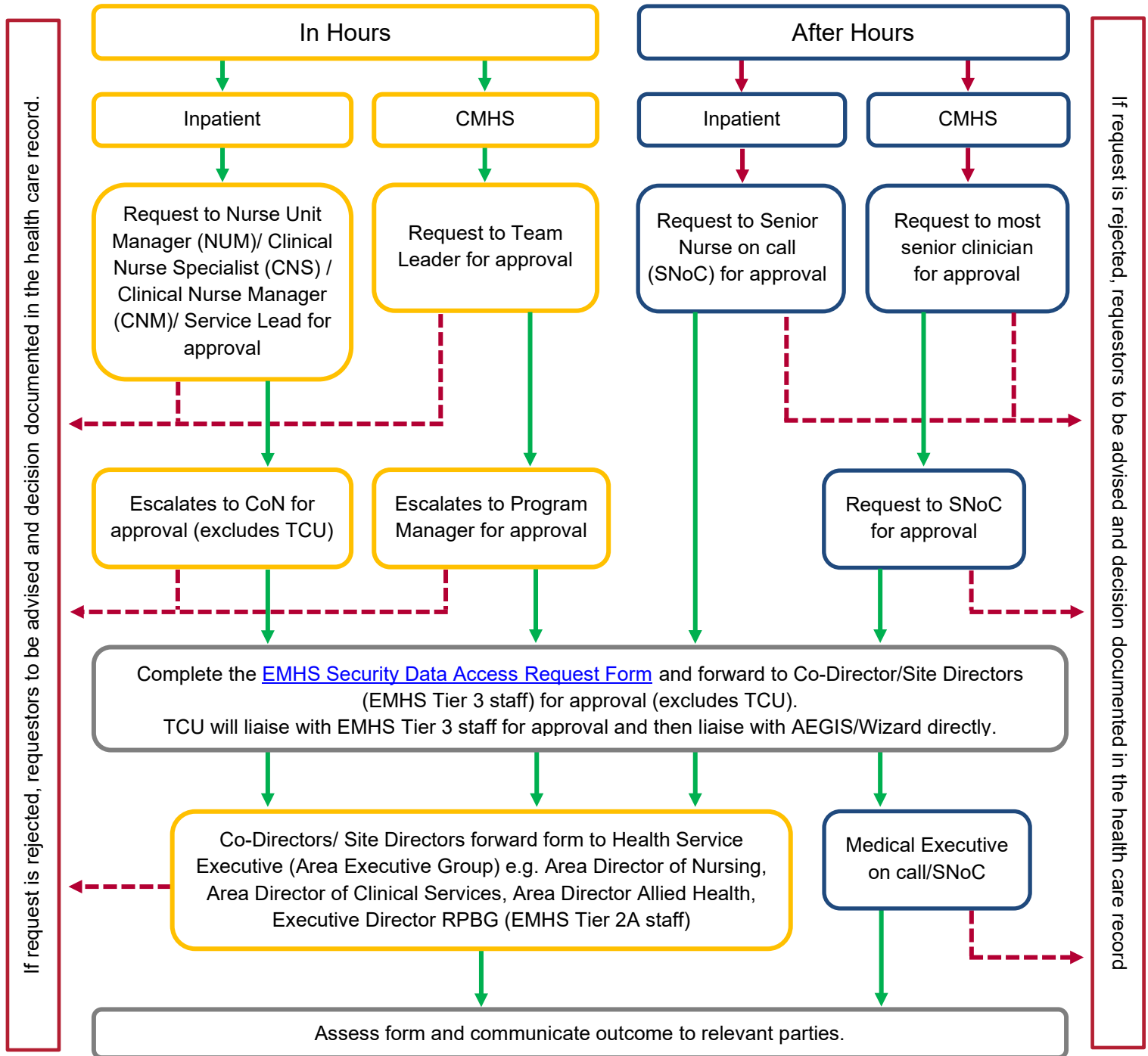
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Procedure

Access, storage and reporting faults

Retrospective access by RPBG staff to CCTV footage:



Legend:	
	Approved
	Rejected

Retrospective access to footage is restricted to authorised persons:

In hours: NUM/CoN/Service Lead

After hours: SNoC/Medical Executive on call

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Procedure - Access, storage and reporting faults cont'd

- Refer to **EMHS Appropriate access and disclosure of Closed Circuit Television (CCTV) footage and other security data Policy** for procedure for non RPBG staff to access CCTV footage.

CCTV location maps and reporting faults

- Stored maps of CCTV camera and monitor locations, including the name of the ward/service/ hospital and Plan number are required for security purposes and to meet the Chief Psychiatrists requirements for authorisation of a hospital.
 - Campus Facilities Managers/Security are to facilitate storage of these maps, e.g. via:
 - Royal Perth Hospital (RPH): Security Access Control System (Honeywell) and RPH SIS MAP (electronic mapping system).
 - Bentley Health Service (BHS): RPH SIS MAP.
- Staff are to report CCTV faults to Security Personnel (excludes TCU) who will help to determine whether it's a fault or direct to Facilities Management who will attend to any repairs. TCU Clinical Nurse Manager (CNM) or Service Lead will contact AEGIS to report faults and request repair / maintenance for guidance.

Use of video monitoring in seclusion (excludes CMHS and TCU)

- Where a seclusion room is authorised for use, CCTV is routinely used to monitor consumers placed in seclusion. However, **CCTV is not a substitute for direct visual observation** (i.e. face-to-face or via an observation window) by a Nurse or Mental Health Practitioner (refer to s. 222, MHA 2014 and **Minimisation and Safe Management of Restraint and Seclusion (for Mental Health Areas) CPS**).
- Sensitivity and discretion are to be used to maintain dignity in regards to toileting and/or other personal needs of the consumer in seclusion. The CCTV monitor may be switched off to allow the consumer privacy as directed by risk assessment.
- In the event that the CCTV monitor is switched off, the Shift Coordinator must continue to assess the consumer's risk and implement appropriately timed direct visual observations in addition to the mandated observations every 15 minutes.
- Staff must also:
 - Inform the consumer when the CCTV monitor is switched on and/or off.
 - Document the reason for the CCTV monitor being switched off and the times that the monitor was switched off and switched back on again in the health care record.
 - Ensure that this information is conveyed in clinical handover.

Notification

- New staff will be notified that CCTV monitoring occurs within the area as part of the local orientation process and are to ensure familiarity with the processes detailed in this SOP.
- Notification signage must be displayed at both the public and WAPOL /ambulance entrances to the area to inform visitors/patients that the area has CCTV monitoring in place. Signs are also placed in areas where CCTV monitoring occurs. These signs are to be large enough to be seen easily and clearly indicate, in writing with a supporting image, that the individual may be monitored.
- Consumer orientation to the area should include, where practical, a verbal notification and explanation of CCTV monitoring. Staff will answer any questions consumers or visitors may have about the purpose of the monitoring and direct any enquires they are unable to answer to the Clinical Nurse Specialist (CNS)/TCU CNM /Nurse Unit Manager (NUM).

Compliance monitoring

A Datix Clinical Incident Management System (CIMS) entry is to be completed for any incidents that do not adhere to this SOP and/or Combined Hazard or Incident Reporting (CHoIR) Online Form and/or Notifiable incidents to Chief Psychiatrist (if required) and is to be monitored by NUM/CNS/Shift Coordinator and CoNs.

The NUM or delegate will audit:

- records of staff to ensure that orientation has occurred relating to CCTV.
- that signage stating the presence of CCTV is visible within the unit/service.

Facilitator

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Authors / acknowledgements

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Transitional Care Unit (TCU) Referral, Admission, Discharge and Transfer Policy through consultation with the Aboriginal Health Strategy Unit and Aboriginal Workforce Engagement Group to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 1566).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Bibliography

1. Government of Western Australia. (2022). *Use of Video Monitoring in Mental Health Inpatient Units Policy*. East Metropolitan Health Service (EMHS). Western Australia.
2. Government of Western Australia. (2021). *Appropriate access and disclosure of Closed Circuit Television (CCTV) footage and other security data Policy*. EMHS. Western Australia.
3. Government of Western Australia. (2019). *Mental Health Advocates Policy*. EMHS. Western Australia.
4. Mental Health Commission. (2014). *Mental Health Act 2014*. Western Australia

Related policies, practice standard, clinical guidelines

RPBG Policies

- Minimisation and Safe Management of Restraint and Seclusion (for Mental Health Areas) Clinical Practice Standard (CPS)
- Save Haven Access and Provision of Care SOP
- Safe and Supportive Observations Policy
- Recognising and Responding to Acute Deterioration Policy
- Recognising and Responding to Acute Deterioration in Mental State Cognition and Behaviour Policy
- Management of Acute Patient Behavioural Disturbance 16years+ Clinical Guideline
- Clinical Handover Policy
- Clinical Care of People Who May Be Suicidal Policy

EMHS Policies

- Use of Video Monitoring in Mental Health Inpatient Units Policy
- Appropriate access and disclosure of Closed Circuit Television (CCTV) footage and other security data Policy
- Mental Health Advocates Policy
- Prevention and Management of Workplace Aggression and Violence Policy
- Psychiatric Service Online Information Service (PSOLIS): communication of risk (Alert) Guideline
- Rights of Patients in Mental Health Policy
- Rights of Carers and other Personal Support Persons Policy

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DoH Policies

- State-wide Standardised Clinical Documentation (SSCD)
- Clinical Incident Management (CIM) Policy
- Safety Planning for Mental Health Consumers Policy
- Principles and Best Practice for the Care of People Who May Be Suicidal
- Safety Planning Procedures for Mental Health Consumers
- Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour
- Information Access, Use and Disclosure Policy
- Information Storage Policy
- Information Security Policy

Other

- [Freedom of Information \(health.wa.gov.au\)](http://health.wa.gov.au)
- [EMHS FOI brochure](#)
- [General Disposal Authority for State Government Information, Authority Number DA 2013-017/1](#)
- [EMHS Delegation and Authorisation Schedule](#)
- Mental Health Act (MHA) 2014
- State Records Act 2000 (WA)
- Surveillance Devices Act 1998 (WA)
- Health Services Act 2016 (WA)
- [The Chief Psychiatrist's Standards for Authorisation of Hospitals under the Mental Health Act 2014](#)
- WA Chief Psychiatrist's Standards for Clinical Care 2015
- Charter of Mental Health Care Principles (MHA 2014)
- [WA Health Patient Confidentiality factsheet](#)

Related forms

- [EMHS Security Data Access Request Form](#)

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021); Standard 1 – Clinical Governance;
Standard 6 – Communicating for Safety

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